

HOW TO OPEN An Assisted Living Residence or Comprehensive Personal Care Home in New Jersey

A step-by-step startup guide — the models, staffing, and how you get paid

Regulatory basis: N.J.A.C. 8:36 (Standards for Licensure of Assisted Living Residences, Comprehensive Personal Care Homes, and Assisted Living Programs) · N.J.S.A. 26:2H (Health Care Facilities Planning Act) · Medicaid MLTSS under NJ FamilyCare

Oversight: New Jersey Department of Health, Division of Long-Term Care Systems, Office of Certificate of Need & Healthcare Facility Licensure

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The Big Picture

New Jersey regulates assisted living under one set of rules — N.J.A.C. 8:36 — that covers three related models, all licensed and surveyed by the New Jersey Department of Health, Division of Long-Term Care Systems. Understanding which model fits your plan is the first and most important step, because the model shapes your building, your staffing, and how you get paid.

An assisted living residence (ALR) is the apartment-style model: private, apartment-style units with a lockable door, a private bath, and a kitchenette, where residents receive coordinated services and can age in place as their needs grow. A comprehensive personal care home (CPCH) delivers the same kind of assisted living services in a more residential, homelike setting — bedrooms rather than apartments, with each unit housing no more than two residents behind a lockable door — which makes it the natural entry point for a smaller operator, and it is the model many former residential health care facilities and boarding homes convert into. The assisted living program (ALP) is different in form: it brings assisted living services to tenants who already live in publicly subsidized housing, rather than housing residents itself.

Two features make New Jersey distinct from many states, and they run through everything below. First, medications may be administered to residents by trained, unlicensed staff working under the delegation and supervision of a registered professional nurse — not merely “assisted” with, as in some states — which means a full-time RN and a trained medication-aide workforce are central to the operation. Second, New Jersey is built around aging in place and managed risk: the rules expect you to help a resident stay as their needs change, and to honor a resident’s informed choices about risk through a written managed-risk agreement. This guide focuses on the ALR and CPCH — the core ownership paths — and is honest about the part everyone asks about: how you actually get paid. It is general educational information, not legal advice; confirm every step, figure, and contact with the Department of Health before you rely on it.

The Three Models — What Each One Means

Model	What it is	Who pays
Assisted Living Residence (ALR)	Apartment-style units (private unit, bath, kitchenette, lockable door) with coordinated services and aging in place. Requires a certificate of need (expedited review available).	Private pay, or Medicaid MLTSS for services + resident income/SSI for room & board
Comprehensive Personal Care Home (CPCH)	Homelike setting; bedrooms with no more than two residents per unit and a lockable door; room, board, and assisted living services when needed. The smaller-operator entry point.	Same as ALR — MLTSS for services + income/SSI for room & board
Assisted Living Program (ALP)	Assisted living services delivered to tenants of publicly subsidized housing, rather than a facility that houses residents itself.	Medicaid MLTSS for services; housing paid through the subsidized-housing program

The Path at a Glance

1. Decide your model — ALR (apartment-style) or CPCH (homelike, smaller) — and set up the business (entity, EIN, insurance, workers' compensation).
2. Contact the Department of Health early to confirm current licensing standards for your model and county (regional licensing teams are listed at the back of this guide).
3. Obtain a certificate of need (CON); New Jersey offers an expedited review for assisted living residences.
4. Secure a location and confirm local zoning, then submit construction plans to the Department of Community Affairs, Health Care Plan Review, before you build or renovate.
5. Recruit your leadership — a licensed assisted living administrator and a registered professional nurse — and plan your trained medication-aide workforce.
6. Submit the license application to the Department of Health with the required fee, and pass the pre-licensure survey and local building, fire, and health approvals.
7. Adopt your policies, assessment tools, service-plan and managed-risk forms, and medication-delegation system.
8. Enroll to participate in Medicaid MLTSS so eligible residents can receive covered services, and plan for your Medicaid-access commitment.
9. Open, admit residents against your assessment and service-planning process, and operate for ongoing Department surveys, reportable-event reporting, and annual renewal.

Part 1 • Choose Your Model and Set Up the Business

Start with the choice between an ALR and a CPCH, because it drives your building. An ALR requires apartment-style units — each with its own bath and kitchenette — which usually means new construction or a substantial conversion and more capital. A CPCH lets you serve residents in bedrooms (no more than two residents per unit, each with a lockable door) in a more homelike building, which is why smaller operators, and conversions of former residential health care facilities or boarding homes, often begin here. Both deliver the same assisted living services and both must meet N.J.A.C. 8:36; the difference is the setting and the cost to build it.

Set up the business properly before you invest in the license. Form your legal entity, obtain your federal EIN, register with New Jersey for taxes and employer obligations, and put workers' compensation and liability coverage in place. New Jersey will evaluate the proposed operator's licensing track record — in New Jersey and other states — as part of licensure, so your ownership and management structure and your history matter from the start.

Part 2 • The Certificate of Need

Unlike many states, New Jersey requires a certificate of need (CON) before you can be licensed to operate an assisted living residence, comprehensive personal care home, or assisted living program. The CON is the state's way of approving that the service is appropriate before the license application proceeds. The important, practical point for assisted living is that New Jersey law permits an expedited certificate-of-need review for these applications, which is less intensive than the full review and does not go before the State Health Planning Board.

Treat the CON as the gate you pass through first: confirm the current CON pathway and forms with the Department of Health's Certificate of Need and Licensing office (contact information is at the back of this guide), and understand any commitments — such as Medicaid access — that attach to your approval, because you will be held to them after you open.

Part 3 • Plan Review and the Physical Plant

New Jersey separates construction plan review from health licensing. Any assisted living residence or comprehensive personal care home with a construction program — whether or not a CON is required — must submit plans to Health Care Plan Review in the Department of Community Affairs, Division of Codes and Standards, for review and approval before construction begins. Before occupying a new or renovated building, you obtain a certificate of occupancy from the municipality and provide the required releases to the Department of Health.

Plan the physical plant around your model. An ALR must provide apartment-style units meeting the space, privacy, bath, and kitchenette requirements; a CPCH must provide residential units housing no more than two residents with a lockable door. Both must meet life-safety and fire requirements, provide accessible common and dining areas, and support aging in place. Getting plan review right early prevents the most expensive kind of rework — the kind you discover after you have built.

Part 4 • The License Application

After you have your certificate of need, you apply to the Commissioner of Health for a license on the Department's prescribed forms, obtained from the Office of Certificate of Need and Healthcare Facility Licensure. You submit a copy of your admission agreement (or a document stating the scope of your services) for review to confirm it does not conflict with the rules or your CON conditions, along with the required documentation of your operator, ownership, and physical plant.

Budget for the licensing fee. New Jersey charges a nonrefundable application fee of \$1,500 plus \$15.00 per licensed bed, and the same fee applies at each annual renewal, subject to the statutory caps. You apply for the maximum number of beds available in your residential units. Confirm the current fee and forms with the Department before you file, since fees and forms change.

A pre-licensure survey confirms your building, staffing, and systems are ready before residents move in. Have your policies adopted, your administrator and registered professional nurse in place, your medication-delegation system established, and your resident records and forms ready when the surveyors arrive.

Part 5 • Leadership, Staffing and the Medication Model

Two roles anchor a New Jersey assisted living operation. The first is a licensed assisted living administrator, responsible for day-to-day management; administrators are licensed through the state's licensing board, and you confirm the current qualification and licensing route before you hire. The second is a registered professional nurse. New Jersey expects an RN presence sufficient to oversee residents' health care plans and — critically — to delegate and supervise medication administration.

This is where New Jersey differs from states that only allow “assistance with self-administration.” Here, trained, unlicensed staff — personal care assistants who have completed the required nurse aide training, and certified medication aides who have completed medication-aide training — may administer medications to residents under the delegation and supervision of the registered professional nurse. That model lets a well-run home meet significant medication needs, but it puts real weight on your RN's delegation, training, and oversight. Build your staffing plan around a full-time RN, a trained medication-aide workforce, and the criminal-background screening New Jersey requires for health care facility staff.

Plan enough qualified staff to make assisted living services and supervision available 24 hours a day, with awake overnight coverage sufficient for your residents' needs. Your staffing is not a fixed number — it rises with census and acuity, which is the practical cost of aging in place.

Part 6 • Policies, Assessment and Managed Risk

Before you open you adopt the written policies that govern resident care and facility operation, and you put in place the assessment and service-planning process at the heart of the rules. Each resident is assessed on admission, periodically, and on a change in condition, with registered professional nurse involvement; from that assessment you build an individualized service plan and a health care plan that say exactly what services, supervision, and health-related care you will provide.

Managed risk is the New Jersey concept that makes aging in place work. When a resident wants to make a choice that carries risk — a mobility choice, a dietary choice, a decision to decline a recommended service — you do not simply say no or discharge. You document the resident's informed choice and the steps you will take to reduce the risk in a written managed-risk agreement, honoring autonomy while protecting health and safety. Your policy manual, assessment tools, service-plan and health-care-plan forms, and managed-risk agreement are the operational backbone the surveyors will look for.

Part 7 • How You Get Paid

This is the question behind every assisted living business plan, and New Jersey's answer has two clean halves: room and board, and services.

Room and board — the rent, meals, and housekeeping — is paid privately, from the resident's own income and savings, or supplemented by the resident's Supplemental Security Income where they qualify. Medicaid never pays room and board in assisted living. Services — the personal care, supervision, medication administration, and health-related assistance — are paid either privately or, for residents who qualify, by New Jersey Medicaid through Managed Long Term Services and Supports (MLTSS) under NJ FamilyCare.

MLTSS is the engine that makes assisted living reachable for residents who are not wealthy, and New Jersey's version has a crucial advantage over some states: it has no enrollment cap and no waiting list. A resident who meets the clinical and financial tests receives services — they are not competing for a limited pool of slots. To qualify, a resident must meet a nursing-facility level of care (generally, needing hands-on help — or supervision for cognitive impairment — with three or more activities of daily living) and the financial limits below.

When a resident is on MLTSS, the arithmetic of room and board is specific: the resident keeps a personal needs allowance of \$140.15 per month and turns the rest of their monthly income over toward room and board, with the balance covered privately or by SSI. Understanding this split — Medicaid pays services, the resident's income pays room and board, and \$140.15 stays with the resident — is the key to pricing your home and explaining costs to families honestly.

New Jersey Assisted Living Funding — The Honest Table

Every figure below is a verified New Jersey figure at the time of writing. Program numbers change annually; confirm current amounts with the Department of Health, NJ FamilyCare, and your County Board of Social Services before you rely on them.

Item	Amount / rule (verify current figures)
What Medicaid MLTSS covers	Assisted living services (personal care, supervision, medication administration, health-related services) — never room and board.
MLTSS caps / waitlist	No enrollment cap and no waiting list — qualifying applicants receive

	services.
Clinical eligibility	Nursing-facility level of care — generally hands-on help, or supervision for cognitive impairment, with 3 or more activities of daily living.
Income limit (2026)	Up to about \$2,982 / month (300% of the SSI federal benefit rate) for the individual; a Qualified Income Trust is used if income is over the limit.
Asset limit (2026)	Under \$2,000 in countable resources for a single applicant (spousal rules differ).
Personal needs allowance	\$140.15 / month kept by the MLTSS resident; the remainder of income is applied toward room and board.
Room & board	Private pay from resident income, supplemented by SSI where the resident qualifies. Not covered by Medicaid.
Private-pay average (2026)	About \$8,234 / month statewide for assisted living (varies widely by county, unit, and care level).
Licensing fee	\$1,500 application fee plus \$15.00 per licensed bed, at application and each annual renewal (subject to statutory caps).

Part 8 • The Medicaid-Access Commitment

New Jersey does not let assisted living serve only private-pay residents. Licensure carries a Medicaid-access commitment, and you should plan for it from day one. An assisted living residence commits that, within 36 months of licensure, at least 20 percent of its residents (measured on a resident-day basis) will be people with nursing-home level-of-care needs; a comprehensive personal care home has a Medicaid-access requirement of its own. Practically, this means you enroll to participate in MLTSS, build relationships with the managed care organizations, and admit and retain Medicaid-eligible residents as part of your census — not as an afterthought.

Track your resident mix against your commitment on a regular cycle, and document it. A home that ignores this obligation until month 30 can find itself scrambling; a home that plans for it prices and markets accordingly and treats MLTSS participation as a core part of the business.

Part 9 • After You Open

A license is the beginning of compliance, not the end. The Department of Health surveys assisted living facilities, investigates complaints, and expects you to report reportable events — certain incidents, injuries, and allegations of abuse, neglect, or exploitation — within required timeframes. Maintain your reportable-event log, your incident records, your drills, and your resident and personnel records so they are always ready for review.

Keep your leadership and workforce current: administrator and RN licenses, staff background rechecks and health screening, medication-aide training and nurse delegation, fire drills, plan and equipment inspections, and assessment and service-plan reviews. Renew your license each year with the required fee, submit and sustain a plan of correction promptly for any deficiency, and run an annual self-assessment against the rules and your own policies so problems surface on your terms rather than a surveyor's.

Part 10 • Finding Residents

Referrals in assisted living come from the people families turn to when a loved one can no longer manage alone. Build relationships with hospital discharge planners and social workers, skilled nursing facilities managing step-downs, home health and hospice agencies, and the county Aging and Disability Resource Connection (ADRC), which is where many families start. Because New Jersey's MLTSS has no waitlist, you can honestly tell referral sources that a qualifying resident can get services — a real advantage worth making known.

Be accurate in everything you publish. New Jersey allows only licensed assisted living residences and comprehensive personal care homes to advertise as “assisted living,” so describe your license truthfully, never claim services you do not provide, and let your managed-risk, aging-in-place philosophy — the promise that a resident can stay as needs change — be the story families remember.

What It Costs to Start — Planning Categories

Startup costs vary enormously between a new apartment-style ALR and a homelike CPCH conversion, so treat the list below as planning categories rather than a quote. The only fixed, verified state figure is the licensing fee; everything else depends on your building, county, and model.

- Certificate of need and licensing — licensing fee of \$1,500 plus \$15 per licensed bed at application and each renewal; plus any professional help with the CON and application.
- Real estate and construction — the largest and most variable cost; an apartment-style ALR costs far more to build or convert than a homelike CPCH.
- Plan review, permits, fire safety, and certificate of occupancy — architectural and code work through the Department of Community Affairs and your municipality.
- Leadership and staffing before opening — recruiting a licensed administrator and a registered professional nurse, and training your medication-aide workforce.
- Insurance — liability, property, and workers' compensation coverage.
- Furnishings, equipment, and systems — resident-unit and common-area furnishings, call/monitoring systems, kitchen, and records systems.
- Working capital — several months of operating reserve to carry payroll and costs while census builds to a sustainable level.

Common Mistakes to Avoid

- Skipping the certificate of need — in New Jersey the CON comes before licensure; do not sign a lease or start construction assuming you can license without it.
- Building before plan review — construction plans must be approved by the Department of Community Affairs first; building ahead invites expensive rework.
- Choosing ALR when CPCH fits — many smaller operators over-build by defaulting to apartment-style units when a homelike CPCH would meet their plan at far lower cost.
- Underestimating the RN and delegation model — New Jersey's medication-administration-by-delegation model depends on real RN capacity; thin nursing coverage is a compliance and safety risk.
- Ignoring the Medicaid-access commitment until it is nearly due — plan MLTSS participation and your resident mix from the start.
- Treating managed risk as a waiver — a managed-risk agreement documents an informed choice and your mitigation; it does not excuse failing to meet a resident's assessed needs.
- Advertising as “assisted living” without the license, or claiming services you are not licensed to provide.
- Inventing funding promises — be precise with families: Medicaid pays services, resident income pays room and board, and the resident keeps the \$140.15 personal needs allowance.

Your First 90 Days After Licensure

10. Finalize and adopt your policy manual, assessment tools, service-plan, health-care-plan, and managed-risk forms, and your medication-delegation system.
11. Confirm your administrator and registered professional nurse are in place and your medication aides are trained, delegated, and competency-checked.
12. Complete Medicaid MLTSS participation steps and establish contacts with the managed care organizations and the county ADRC.
13. Run your admission process for the first residents — assessment, service plan, health care plan, admission agreement, and rights — exactly as your policies require.
14. Stand up your logs and calendars — reportable events, incidents, fire drills, inspections, training, and a compliance calendar with every recurring due date.
15. Build referral relationships with hospitals, skilled nursing facilities, home health and hospice agencies, and the ADRC.
16. Schedule your first quality review — look at incidents, medication records, and grievances, and correct what you find before a surveyor does.

Local Call List — Essex County (Newark)

These are the local offices a new operator in the Newark area actually needs. Phone numbers are verified at the time of writing; confirm before you rely on them.

Office	Purpose	Contact
Essex County Dept. of Citizen Services — Division of Family Assistance & Benefits	Medicaid / Board of Social Services; MLTSS financial eligibility	973-395-8000; 320 University Ave, 2nd Fl, Newark 07102
Essex County Division of Senior Services (ADRC)	Aging & Disability Resource Connection; MLTSS assessment referral	973-395-8375; 465 Dr. MLK Jr. Blvd, Suite 102, Newark 07102
City of Newark — Office of Uniform Construction Code (City Hall)	Building permits, construction code, certificate of occupancy	973-733-4311; 920 Mayor Kenneth A. Gibson Blvd, Newark 07102
Newark Dept. of Health & Community Wellness	Local health, environmental health, food service	973-733-7604; 110 William St, Newark 07102
Adult Protective Services (Essex)	Report suspected abuse, neglect, or exploitation of a community adult	Route via ADRC 1-877-222-3737; DOH complaint hotline 800-792-9770

Local Call List — Hudson County (Jersey City)

The Newark and Jersey City lists together cover New Jersey's two largest cities. Phone numbers are verified at the time of writing; for the Jersey City construction and health offices, confirm the current direct line with the city.

Office	Purpose	Contact
Hudson County Dept. of Family	Medicaid / Board of Social Services;	201-420-3000; 257 Cornelison

Services — Welfare Division	MLTSS financial eligibility	Ave, Jersey City 07302
Hudson County Office on Aging (ADRC)	Aging & Disability Resource Connection; MLTSS assessment referral	201-369-4313; 830 Bergen Ave, Suite 3B, Jersey City 07306
Jersey City — Division of Construction Code Official	Building permits, construction code, certificate of occupancy	One Jackson Square (364 MLK Dr), Jersey City 07305; portal jcnj.org/permitportal
Jersey City Dept. of Health & Human Services	Local & environmental health, food service	Division of Environmental Health, City of Jersey City (confirm direct line)
State construction help / Adult Protective Services	Code questions; report abuse/neglect/exploitation	DCA Code Assistance 609-984-7609; APS via ADRC 1-877-222-3737

Statewide Contacts

Verified at the time of writing. Programs and numbers change — confirm before relying on them.

Agency / purpose	Contact
NJ DOH — Certificate of Need & Licensing, Health Facilities	609-292-6552; PO Box 358, 120 South Stockton St, 3rd Fl, Trenton 08608
NJ DOH — Licensing Intake Unit / Renewals	609-376-7800; portal dohlicensing.nj.gov ; cnandlicensingrequests@doh.nj.gov
Licensing standards — regional teams	Team A (Bergen, Hudson, Mercer, Morris, Passaic, Somerset, Sussex, Warren) 609-292-6552; Team B (Burlington, Gloucester, Hunterdon, Middlesex, Monmouth, Ocean) 609-633-9042; Team C (Atlantic, Camden, Cape May, Cumberland, Essex, Salem, Union) 609-292-7228
NJ DOH — Long-Term Care Complaint & Surveillance	609-633-8991
Health care facility complaints / reportable events (24/7 hotline)	800-792-9770 (select option 1 for long-term care)
Certified Assisted Living Administrators (CALA) / Administrator Licensing Board	609-633-9706
Certified Medication Aide (CMA)	609-633-8990; DOH-cma@doh.nj.gov
Certified Nurse Aide (CNA) & Personal Care Assistant (PCA) certification	609-376-7755
Enforcement and Certification	609-984-8128 or 609-376-7890
Criminal Background Investigations (health facility staff)	866-561-5914; PO Box 359, Trenton 08625-0359
NJ DCA — Health Care Plan Review / Codes & Standards (construction plans)	609-292-7899; Code Assistance 609-984-7609; PO Box 802, Trenton
Medicaid MLTSS / NJ FamilyCare — apply via county ADRC or Board of Social Services	Aging & Disability Resource Connection 1-877-222-3737; DDS (under 21) 1-888-285-3036
NJ Long-Term Care Ombudsman	nj.gov/ooie ; PO Box 852, Trenton 08625-0852 (resident advocacy; abuse reporting)

Glossary

ALR — Assisted living residence — the apartment-style model under N.J.A.C. 8:36.

CPCH — Comprehensive personal care home — the homelike model (bedrooms, up to two residents per unit, lockable door); a common smaller-operator entry point.

ALP — Assisted living program — assisted living services delivered to tenants of publicly subsidized housing.

CON — Certificate of need — New Jersey approval required before licensure; an expedited review is available for assisted living residences.

MLTSS — Managed Long Term Services and Supports — New Jersey's Medicaid managed-care program that pays for assisted living services (not room and board), with no cap or waitlist.

NFLOC — Nursing-facility level of care — the clinical standard (generally help or supervision with 3+ ADLs) for MLTSS eligibility and the ALR access commitment.

PNA — Personal needs allowance — \$140.15/month kept by an MLTSS resident; the rest of income goes toward room and board.

Assisted living administrator — The licensed individual responsible for day-to-day management of the facility.

Medication administration by delegation — New Jersey's model in which trained, unlicensed staff administer medications under registered professional nurse delegation and supervision.

Managed-risk agreement — A written agreement honoring a resident's informed choice that involves risk, while documenting steps to protect health and safety — central to aging in place.

Aging in place — Enabling a resident to remain as needs change, to the extent they can be safely met, through assessment, service planning, added services, and managed risk.

ADRC — Aging & Disability Resource Connection — the county “front door” for long-term-services information and MLTSS assessment referral.

A Final Word on Compliance

New Jersey's assisted living rules exist to protect vulnerable adults, and the state holds licensed operators to them through certificate of need, licensure, survey, and reportable-event oversight. Treat compliance as the foundation of your business, not a hurdle: adopt strong policies, keep your administrator and nursing leadership current, honor residents' rights and their informed choices through managed risk, meet your Medicaid-access commitment, and report and correct problems promptly.

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